Retention report

Omzo GLP-1 telehealth · synthetic dataset, Jan–Sep 2026 · all figures computed from the project's data and metrics

Summary

- 80% of new customers are still active one month after signup, 52% at month 3 and 37% at month 8 (pooled across observable cohorts).
- Early loss is concentrated in the monthly plan (71% at M1). Prepaid customers show a step at their first renewal (100% at M2 to 58% at M3).
- Channel matters: M3 retention ranges from 45% (TikTok) to 59% (Organic / referral).
- Failed payments end 1,752 subscriptions (10% of all cancels). Dunning already recovers 59% of failed renewals.
- Three retention actions (check-ins, dunning, pause) add ~$652K to monthly operating cash flow by Jun 2027 in the base plan. The downside case only reaches 2x if they work.

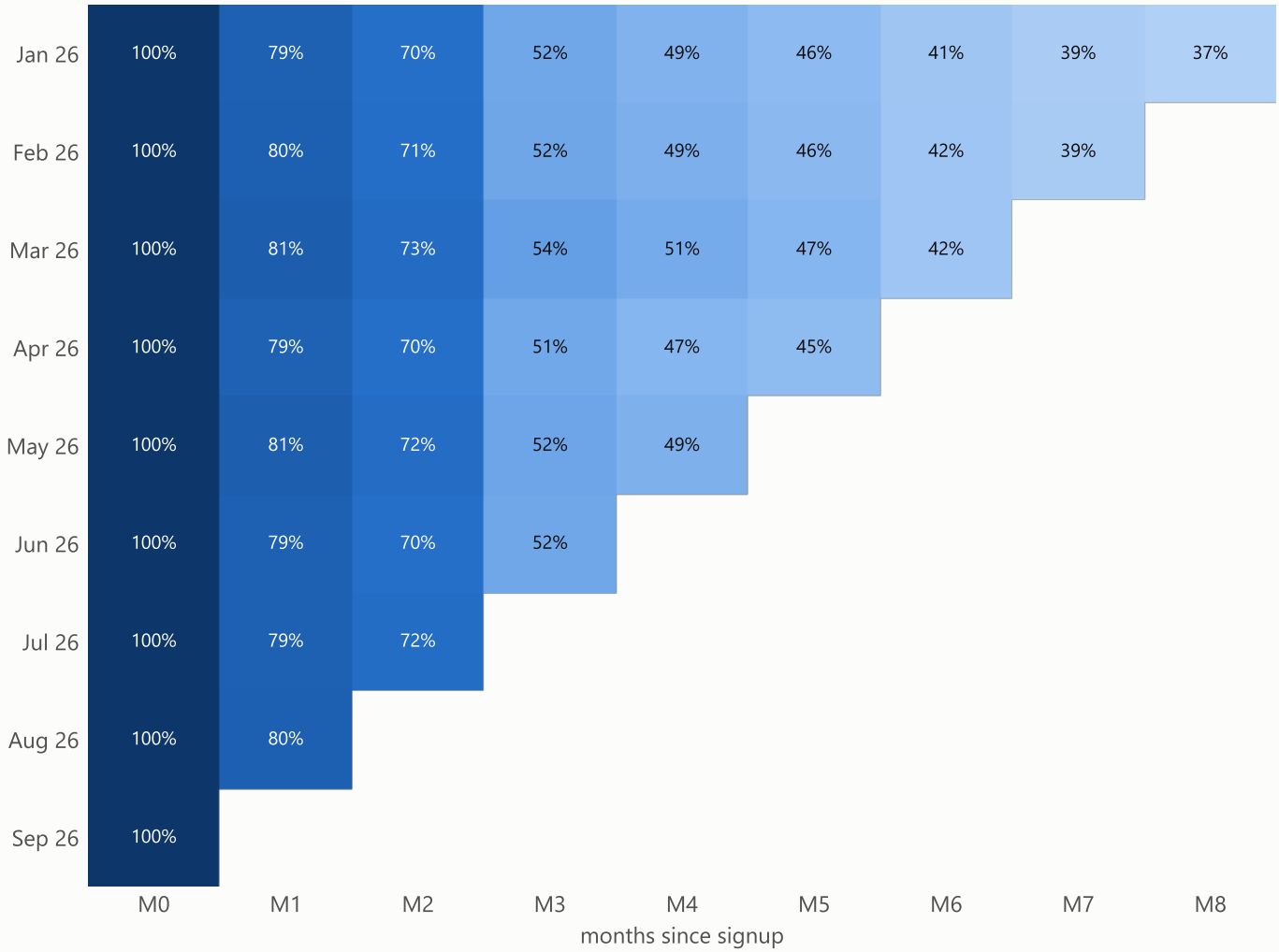
Definitions and handling rules

- Retention (month k): share of a signup cohort active in calendar month signup + k. Active = a successful payment covers that month (a prepaid charge covers 3 months) OR the customer is in an active pause. Month 0 is 100% by construction.
- Churn (monthly): customers active last month who are not active this month (cancelled, or a failed payment not recovered), divided by last month's actives. Pauses count as active, so a pause is not churn.
- Renewal rate: customers whose paid term ended last month who paid again this month, divided by those due (customers who paused instead are excluded from the denominator).
- Pauses: 2,674 one-month pauses in the data. Paused customers stay in the retention numerator; the paid-share metric separates paying from paused.
- Reactivations: 1,286 win-backs. A returning customer stays in the ORIGINAL signup cohort, so a cohort's retention can rise slightly in later months.
- Failed payments: 7.9% of renewal charges fail at least once; retries run on days 1, 3, 5. Recovered = active; unrecovered = involuntary churn ('payment_failed').
- Right-censoring: months after Sep 2026 are not yet observable. They are blank (NULL), never 0. Pooled curves use only cohorts observable at that month, so later points rest on fewer, older cohorts.

Caveat: the data is synthetic, generated from stated assumptions. 'Supported by the data' below means visible in this dataset; real data would be needed to confirm the patterns.

Cohort retention

Cohort retention (active = paid or paused; blank = not yet observable)

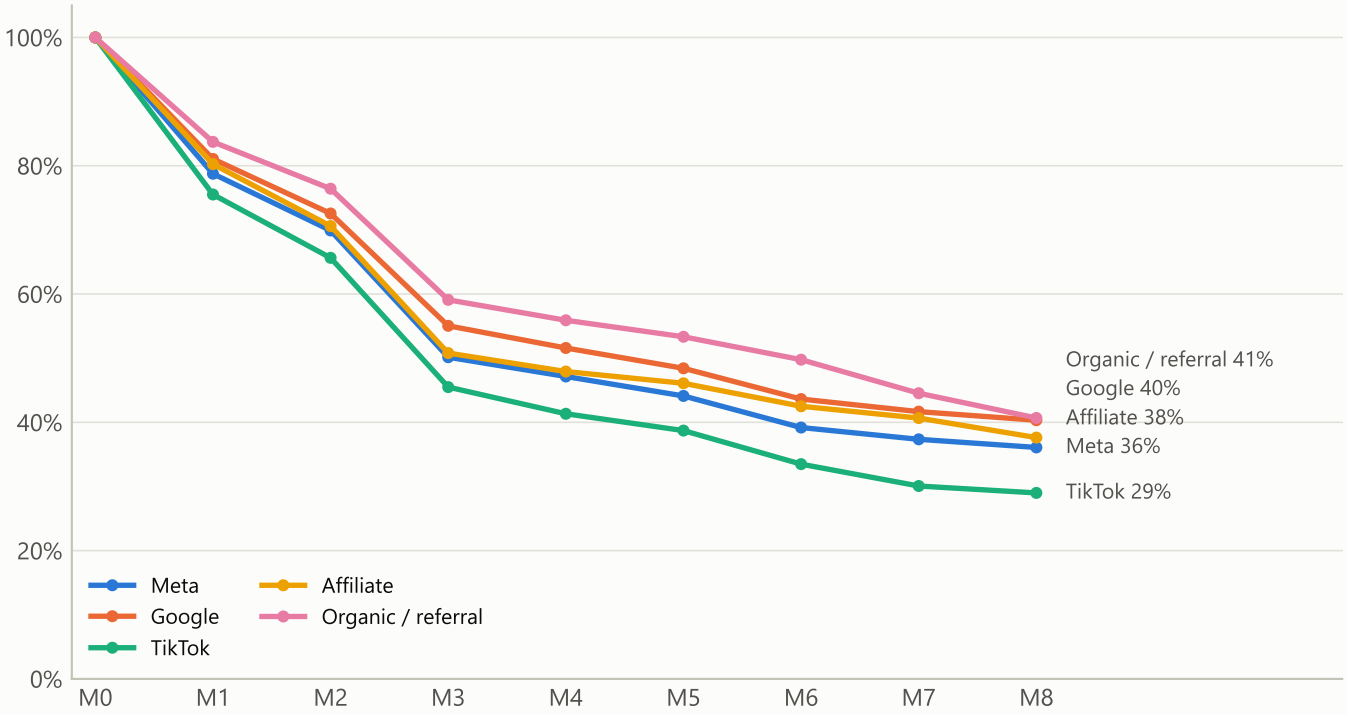


Observations

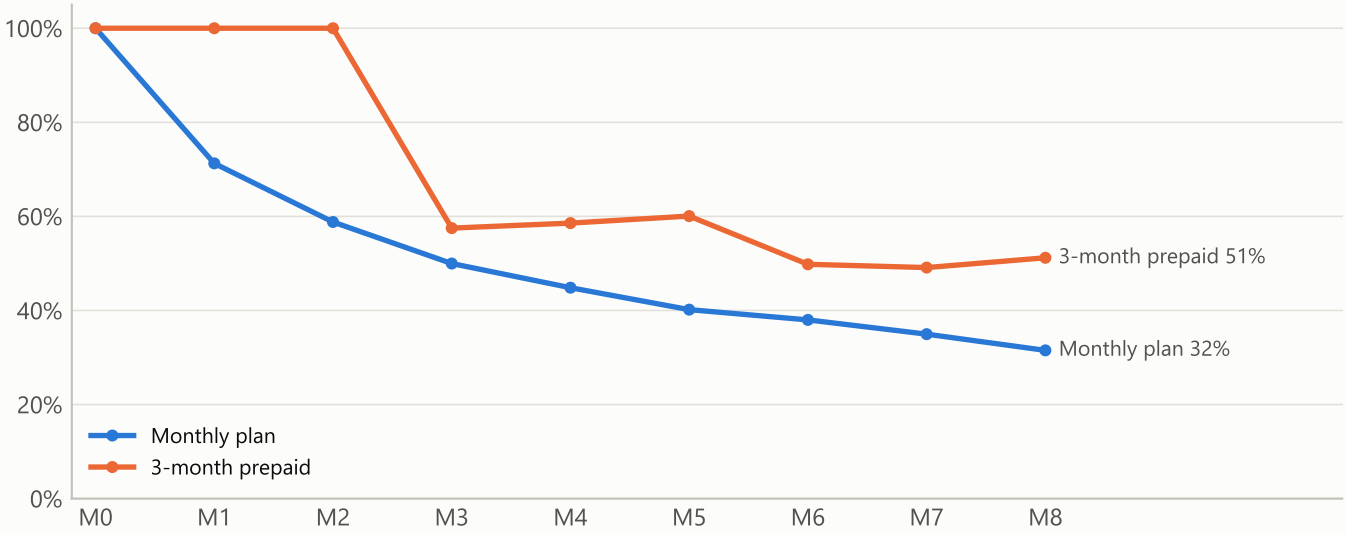
- 1. The steepest losses come first: M1 80%, M2 71%, M3 52%. After M3 the curve flattens to ~3.6%-pt per month (M6 42%).
- 2. Cohorts are stable: M1 retention is 79%–81% for every cohort Jan–Aug. Growth has not come with worse-quality customers so far.
- 3. Portfolio churn is falling as the base matures: monthly churn 20.8% (Feb 26) to 14.5% (Sep 26); renewal rate 70% to 79%. That is a mix effect (more long-tenure customers), not proof that early churn improved.

Retention by channel and plan

Retention by acquisition channel



Retention by plan: prepaid cannot churn before its first renewal (M3)



Observations

- 4. Channel gap: Organic / referral retains 13.6%-pts more than TikTok at M3 and 16.3%-pts more at M6. M1/M3/M6 by channel: Meta 79%/50%/39% · Google 81%/55%/44% · TikTok 76%/45%/33% · Affiliate 80%/51%/43% · Organic / referral 84%/59%/50%.
- 5. Plan: the monthly plan loses 29% of customers after the first month. Prepaid holds 100% to M2, then 58% renew; at M8, prepaid 51% vs monthly 32%.

Why customers leave

(a) SUPPORTED by the dataset

- Churn spike in months 1–2: monthly-plan churn is 29% in month 1 and 17% in month 2, vs 10% in month 4 and 5% in month 6.
- Channel differences: M3 retention 45% (TikTok) to 59% (Organic / referral). Paid-social customers churn fastest; referrals slowest.
- Involuntary churn: 4,446 renewals (7.9%) failed at least once; 59% were recovered. 1,801 were lost, ending 1,752 subscriptions (10% of cancels).
- Refund-driven exits: 2,112 cancels (12%) follow a refund, mostly on first orders. Chargebacks block a further 233.
- Stated reasons for voluntary cancels (14,064): price 35%, side effects 25%, other 20%, reached goal 20%.
- Medication-driven dose/cost: dose steps up at tenure months 3 and 5 (cost x1.25 and x1.5), but there is NO matching churn spike (monthly-plan churn 17% into month 3, 10% into month 5, on trend). Customer price never changes with medication cost. In this data the medication effect is on margin (contribution per order fell to its low at the May cost peak), not on retention.

(b) HYPOTHESES needing investigation

- Side effects: 'side effects' is a stated reason, but the data cannot say which effect, when, how severe, or whether dose timing triggers it. Needs: symptom check-in data and clinician notes by tenure.
- Expectations: early weight-loss pace below expectations may drive month-1–2 exits and 'price' reasons. Needs: onboarding survey, weight-trajectory vs churn.
- Delivery issues: cold-chain delays or damaged shipments could trigger cancels. Needs: carrier scans, temperature excursions, reship rate joined to churn.
- Service experience: slow clinician or support responses. Needs: ticket response times and CSAT by customer.
- Channel promise mismatch: promo-heavy social ads may over-promise. Needs: creative-level retention and promo vs non-promo cohorts.
- 'Reached goal' exits may be healthy completions rather than failures. Needs: weight-goal tracking and a maintenance offer.

Recommended actions

Each action: A/B design, primary metric, sample/duration (alpha 5%, power 80%), and estimated effect from the Levers sheet

1. Titration and side-effect check-ins with clinician review (lever b1)

Design: randomise new monthly-plan customers 50/50. Treatment = proactive clinician check-ins at weeks 1, 2, 4 and before each dose step, with symptom triage; control = current care.

Primary metric: M3 retention (monthly plan), baseline 50%; the lever's 15% cut in month 1–3 churn implies 56.1%. Guardrails: adverse-event reports, clinician-initiated stops (must not fall).

Sample/duration: 1,027 per arm; at ~2,768 new monthly-plan customers/month, enrolment takes ~0.7 months plus a 3-month read: ~4 months.

12-mo contribution LTV +$43 per new customer (+6.3%); monthly cash flow +$238K by Jun 2027 (bridge).

2. Dunning: smarter retries, card updater, SMS/email reminders (lever b2)

Design: randomise failed renewals 50/50 to the new dunning sequence vs current retries.

Primary metric: recovery rate of failed renewals, baseline 59% (assumption 60%) vs target 75%. Secondary: involuntary churn, complaints.

Sample/duration: 149 failed renewals per arm; ~494 failures/month means ~0.6 months to enrol, read within 1 week of each failure.

12-mo contribution LTV +$27 per new customer (+4.0%); monthly cash flow +$218K by Jun 2027 (bridge).

3. Pause option instead of cancel (lever b3)

Design: in the cancel flow, randomise 50/50 to a 1-month pause offer vs the standard flow. The pause must be clinically appropriate (clinician confirms it is safe to interrupt and how to restart).

Primary metric: share of cancel-flow entrants active 60 days later; baseline ~6% (win-backs) vs ~18% if 20% pause and 60% resume.

Sample/duration: 112 per arm; ~1,563 voluntary cancels/month means ~0.1 months to enrol plus a 2-month read.

12-mo contribution LTV +$42 per new customer (+6.1%); monthly cash flow +$195K by Jun 2027 (bridge).

Recommended actions (continued)

Each action: A/B design, primary metric, sample/duration (alpha 5%, power 80%), and estimated effect from the Levers sheet

4. Onboarding expectation-setting (not in Levers: no cash credit taken)

Design: randomise new customers to a structured onboarding (realistic weight-loss timeline, side-effect guide, what to expect at each dose) vs current welcome flow.

Primary metric: M1 retention (monthly plan), baseline 71%; minimum effect worth detecting +3pp (74%).

Sample/duration: 3,449 per arm: ~2.5 months to enrol plus a 1-month read.

Effect: not modelled in the Levers sheet, so the plan takes no credit; upside only.

5. Delivery tracking and proactive delay alerts (not in Levers: no cash credit taken)

Design: randomise by shipment week (cluster) to proactive tracking SMS + delay/temperature alerts with automatic reship vs current notifications.

Primary metric: delivery-related tickets per 100 shipments; secondary: M2 retention and reship cost.

Sample/duration: tickets are frequent, so ~4–6 weekly clusters per arm (6–8 weeks) detect a 20% ticket reduction; a retention effect needs the larger sample above.

Effect: not modelled in the Levers sheet; no credit taken. It also produces the data needed to test the delivery hypothesis.

Clinical-suitability guardrail

Retention actions serve patient outcomes, not retention for its own sake. No incentives to stay on medication against clinical advice. No discounts or pressure that discourage a clinically indicated stop. No bypassing eligibility screening to raise conversion or retention. Check-ins and pauses are clinician-led; patients can always cancel in one step. Every test tracks safety guardrail metrics (adverse events, clinician-initiated discontinuations, complaints) and stops if they worsen.

How the estimates were made

LTV effects: 12-month discounted contribution LTV from the fitted retention curves (src/ltv_projection.py), customer-weighted across channels, with each lever's churn change applied at its full Levers value. Cash effects: the workbook's doubling bridge (target month Jun 2027, levers added in fixed order, base case). Sample sizes: two-proportion z-test.